Respiratory block TBL year 3 (2024)

1) Decrease physiological dead space (to decrease the gap/difference between minute ventilation and alveolar ventilation: copy paste from amboss)

2) Ipratropium (bronchodilator that's good for COPD and has the least possibility to cause arrhythmia)

3) Oral montelukast (mom doesn't want ICS for daughter)

4) Decrease FVC, FEV1 and normal FEV1% and PEFR (is asbestos/restrictive)

5) Dried ivy Herbal extract (for 5 year old with dry cough)

6) Blockage of histamine receptors (not imp in asthma prevention/management)

7) Cobolymon** used in acute exacerbation (Incorrect about immunologic)

8) Gas exchange become diffusion limited in sternuous exercise

9) Hb affinity for O^2 decrease qnd curve shifts to the right (in systemic circulation)

10) ABG collected about 1mL from radial artery and proccessed in 20 minutes

11) FRC (amount of air found after normal expiation)

12) At the end of normal expiration (intrapleural nearest to zero)

13) bronchiectasis (necrotizing pneumonia and purulent sputum + decreased fev1/fvc ratio)

14) RELATED TO PREVIOUS Q) what's the histopathology noticed in this;
_ Answer —> mixed acute and chronic inflammatory cells found

15) Usual interstitial pneumonia (diffuse interstitial damage, no mass)
16) RELATED TO PREVIOUS Q) What's the histopathology noticed in this:
_ Answer —> patchy fibrosis and honey-comb fibrosis and temporal heterogeneity

17) What causes decreased airway resistance: VIP

18) Carotid bodies (person had cyanosis and had oxygen therapy and he then had decreased RR, which receptors is less stimulated now, *hypoxia affects peripheral chemoreceptors*)

19) Irritant receptors (for cough reflex and found in trachea)

20) A patient with diabetic ketoacidosis has Kussmaul's breathing: The picture same in the lecture